

B12

Essential for:
Nerve function
Red blood cells

Na/K

Essential for:
Sodium
Potassium
Electrolytes

D

Essential for:
Musculoskeletal
Immunity

Zinc

Essential for:
Wound healing
Hair, Nails, Skin
Immunity
Digestion

Mg

Essential for:
Muscles & Cardiac
Mental anxiety
Potassium



Absorbing the good stuff

Ever felt unusually tired, crampy, or “off”? Research has looked at how ileostomies and colostomies affect the body’s mineral and electrolyte balance, and the findings are worth knowing.

Sodium and potassium are a big concern, according to a 2023 review in the *Journal of Clinical Medicine*.

The study found that people with high-output ileostomies (1,500–2,000 mL a day) are prone to losing sodium and potassium along with fluid. Another study measuring blood levels before and after surgery found a real, measurable drop in potassium (and some drop in sodium) in ileostomy patients in the early weeks after surgery, more so than in colostomy pa-

tients. This is part of why staying hydrated with an electrolyte-replacement drink, not just plain water, matters so much for ileostomates.

“I try to monitor levels with my providers,” said LOA member Amy. “I’m consistently low on vitamin D, iron, and B12. That’s the reality of Crohn’s Disease and living with an ileostomy. My providers recommend liquid or sublingual supplements which are better absorbed for me.”

Calcium, zinc, iron, and magnesium matter too. The same 2023 study found that people with inflammatory bowel disease face a risk of calcium deficiency, which can contribute to weakened bones

(osteoporosis). Low zinc and low iron were both linked to a lower quality of life — more fatigue, more digestive discomfort. And magnesium deficiency showed up more often in people with Crohn’s disease, especially those who’ve had a larger portion of their small intestine removed.

If you haven’t had your levels checked recently, or if you’ve been feeling unusually fatigued or crampy, it’s worth asking your doctor for a simple blood panel. It’s a quick way to catch a problem before it becomes a bigger one.

Vitamin B12 is worth monitoring. B12 is absorbed in the terminal ileum — the very end of the small intestine, which is often

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Vitamins, Minerals

continued from page 1

removed or bypassed in ileostomy surgery.

A 2023 research review found that low B12 was strongly linked to poorer mental health and quality of life in ostomates. The catch is that your liver stores several years' worth of B12, so a deficiency can sneak up on you long after surgery, with symptoms like fatigue, tingling in the hands and feet, balance problems, or memory trouble. The good news, from a study of long-term ileostomy patients with IBD only about 5% of patients in that study were deficient, and the real risk factor was additional bowel surgery or ongoing small bowel inflammation, not the ileostomy by itself. Still, if you have any of those symptoms, ask your doctor for a simple blood test — it's an easy fix once caught, whether through injections, a nasal spray, or high-dose oral supplements.

Fat-soluble vitamins A, D, E,

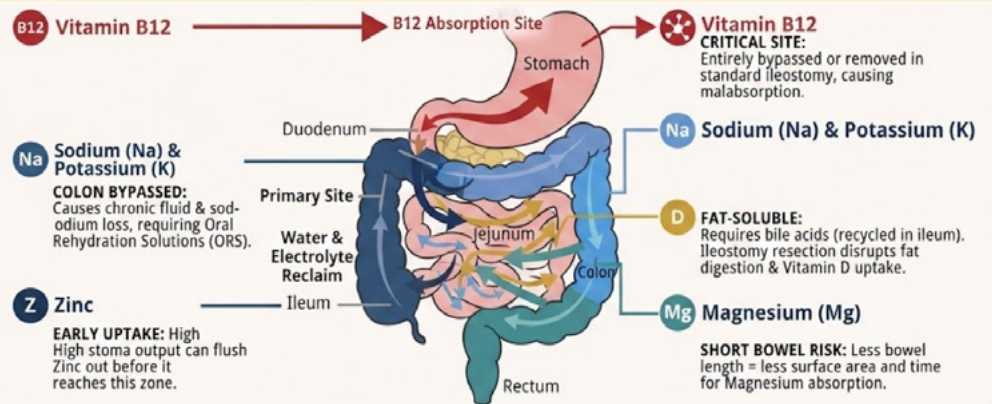
and K deserve attention too. Because these vitamins depend on healthy fat absorption, and because vitamin K is normally supplied in part by gut bacteria in the colon, ileostomy and colostomy patients can run low — vitamin K2 deficiency in particular has been documented in this group. Folate and other B vitamins, also partly produced by gut bacteria, can dip for the same reason.

The bottom line: researchers agree these are real, documented

risks — but there aren't yet formal screening guidelines written specifically for ostomates. That makes it worth a conversation with your doctor about periodic bloodwork, especially if you're feeling unusually tired, foggy, or "not quite right."

Sources: Dereń et al., *Journal of Clinical Medicine* (2023); Jayaprakash et al., *International Journal of Colorectal Disease* (2004); North Central OK Ostomy Chapter newsletter. Sources: Ganguly et al., *IOSR Journal of Dental and Medical Sciences* (2014); United Ostomy Associations of America.

VITAMINS & MINERALS: Viama NG: Normal Absorption vs. Ostomy Impact What Every Ostomate Should Know



KEY TAKEAWAYS FOR OSTOMATES

- Bypass removes key absorption sites.
- Ileostomies carry higher risks than colostomies.
- Fatigue, cramps, and output changes are key warning signs.
- Consult your clinician for routine annual bloodwork.

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Friend of LOA Theresa Johnson, of Stoma Genie, met with Rep. Sheri Scott Biggs in Washington DC to discuss the importance of choice in ostomy products for Medicare recipients.

Ostomy choice relies on *your* voice

If you rely on Medicare for ostomy or urological supplies, a change is coming that could limit your choices — and access.

According to the UOAA, in November 2025, the Centers for Medicare & Medicaid Services decided, to fold individualized, doctor-prescribed products into a Competitive Bidding Program.

Instead of choosing from suppliers who know your needs, Medicare beneficiaries will soon be limited to just seven urological suppliers and eight ostomy suppliers — period. No regional flexibility, no local relationships, just a handful of national mail-order contractors serving the entire country.

“Not only will local relationships be eliminated, the reimbursements will be driven down resulting in lesser quality products being delivered, essentially a race to the bottom of the barrel,” said Joe Frey, owner of [Triumph Home Health Supplies](#) in Lincoln.

“This situation is not good for people who use Medicare to reimburse ostomy supplies,” said Theresa Johnson, owner of [Stoma Genie](#) a longtime LOA supporter. “They said people using Medicare could only obtain supplies from eight distributors (who will bid on contracts). That has now been reduced to six distributors.” Johnson has visited with Congressional leaders in Washington D.C. twice over the issue.

Lincoln Ostomy Association aligns with Theresa and UOAA in opposition to this direction. Ostomy supplies aren’t interchangeable commodities like bandages or gloves — pouching systems, barriers, and accessories are fitted to each person’s stoma, skin, and lifestyle. When Congress created the Competitive Bidding Program in 2003, it deliberately left these products out for exactly that reason. CMS’s new “Remote Item Delivery” program reverses that protection, and contracts are set to be awarded in 2027, limiting patient access starting in 2028.

The good news: decisions are still being made

[Nebraska’s members of Congress](#) are campaigning and holding local events. That makes this the perfect window to tell our story — in person, by phone, or by letter.

Join us for a meeting!

Lincoln Ostomy Association

2-4 p.m.

8320 South St. Aldersgate Church, Lincoln, NE

September 13

Annual Potluck &

Michele Hagenson, MSN, RN, CWON

Recognizing the Symptoms to

Plug the Leak on B12 Loss

December 6

Special Ostomy Panel with WOCNs



Founded in 2003, Lincoln-based Triumph Home Health Supplies key product categories are ostomy, catheters and other health home supplies. The company resides on 4540 N 56th Street near the intersection of Cornhusker Highway and Superior. Joe Frey, left, is owner and a newly-instated Lincoln Ostomy Association. Back Row: Joe Frey, Gary (the dog), Jen Huigens, Todd Davison, Jennifer Julian, Laura Frey Front Row: Pam Sniff, Andrea Acosta, Lindsay Lowe

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When a bag leaks...

Your ostomy pouch, and its seal, is what stands between output and your body. When it fails, stool or urine can find its way underneath and cause a leak.

Most leaks trace back to a handful of causes.

In some cases, the barrier opening doesn't fit your stoma. Your barrier should fit snugly around your stoma. If the opening is too large, your peristomal skin is exposed to output, compromising adhesion of the barrier and opening a path for leaks.

Your skin isn't fully dry at application, moisture can prevent adhesion. Even a small amount – from a shower, sweat, or cleaning the area around the stoma during a bag change – can prevent the seal from forming. Skin folds, the area around the belly button, and any place moisture collects are especially vulnerable.

If your peristomal skin is irritated, adhesion suffers. Irritation can create an uneven surface that can worsen with each change as the barrier pulls on already-sensitive tissue.

If your stoma is flush, retracted, or sits in a skin fold, output can spread sideways under the barrier instead. In these situations, the barrier may not be appropriately matched to your stoma's profile.

You have a wealth of resources to help. Local ostomy nurses

Gastroenterology Specialties P.C.

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specialize in preventing leaks. Lincoln, in particular, is blessed with nurses that hold years of service and have seen it all. Tips from fellow ostomates are often shared at Lincoln Ostomy Association meetings. Get to know us, we're in the same boat!

No surprises Medicare

If you receive ostomy supplies through Medicare, a Nebraska law passed in 2024 may provide an important protection when you use a nonparticipating Medicare supplier.

Under Nebraska law, a nonparticipating supplier generally cannot balance-bill a Nebraska Medicare beneficiary for covered durable medical equipment or supplies—including supplies that fall within Medicare's covered supply categories—unless the patient agrees in writing beforehand to pay the additional amount and pays that amount before receiving the supplies. The law took effect July 19, 2024.

This means it is worth asking questions before placing an order:

“If there is an additional charge, can I get the amount and terms in writing before ordering?”

What will Medicare pay, and what amount will you be responsible for?

The law does not mean that Medicare beneficiaries never have out-of-pocket costs. Medicare deductibles, coinsurance, and other applicable patient responsibilities can still apply. The purpose of the Nebraska law is to provide greater transparency and protect beneficiaries from unexpected balance billing by nonparticipating suppliers.

For 2026, the Medicare Part B deductible is \$283, followed generally by 20% coinsurance for covered services and items.

A simple tip: Before ordering your next shipment, ask the supplier about Medicare participation and any amount you may have to pay. Getting the information in writing can help prevent unpleasant surprises later.

CAUTION: This article is for general education and is not legal or insurance advice. For questions contact Medicare or your insurance/Medigap plan.



LOA says sleep well friends

A good night's sleep can take some adjustment for those with ostomies. The good news is that nighttime routines become easier once you learn how your stoma behaves.

One useful habit is to empty your pouch before going to bed. Starting the night with an empty pouch gives you more capacity and may reduce the chance of waking up because the pouch has become full. UOAA has also noted that avoiding eating too close to bedtime may help minimize nighttime pouch leakage for some people.

Everyone's digestive system is different, and what works for one person may not work for another. Rather than automatically avoiding food for a particular number of hours, pay attention to whether late meals consistently cause increased output, gas, or a fuller pouch during the night.

For people with an ileostomy, hydration remains important even though getting up repeatedly to empty the pouch can be frustrating. Instead of concentrating all your fluids late at night, UOAA recommends spreading

fluid intake throughout the day. Your healthcare professional can help determine the right amount for you.

Finally, remember that an occasional difficult night doesn't mean you're doing something wrong. Living with an ostomy involves learning your body's patterns. With time, many people



develop a predictable routine allowing them to sleep comfortably.

Sources: United Ostomy Associations of America, Diet + Nutrition resources and ostomy education materials.

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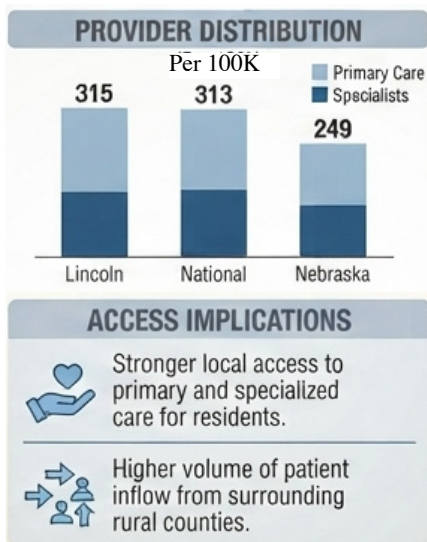
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Norfolk resident blessing others

Tina Collison of Norfolk said an ostomy was never something she imagined would become part of her life.

Tina battled Crohn's disease for years. Her health journey included treatments and surgeries, but eventually her colon stopped functioning leading to an ostomy.

The emotional impact was enormous she said in an [interview with KMTV 3 News Now](#).

She said she worried about what people would think and whether she could continue doing the things she loved. As a fitness instructor at the Norfolk Family YMCA, she spent years helping others stay active and healthy. She said she was afraid that the ostomy would change how people saw her—and how she saw herself.

Tina found ways to become comfortable with her new reality. She continued teaching fitness, eventually teaching 11 classes a week at the YMCA.

Then, Tina turned her experience into Crafting for a Cause, bringing people in the Norfolk community together to make handmade covers for ostomy pouches. A small piece of fabric to make a practical medical device feel a little more personal—and a little less intimidating.

Norfolk's was exactly what she hoped for. People showed up.

They brought their time, their creativity and their willingness to help strangers they might never meet. Tina said it best: "No matter what it is, Norfolk shows up."

Her story is a reminder that sometimes life doesn't return to the way it was before surgery. Sometimes you build something new, something that allows you to help someone else.

Source: [KMTV Video YouTube](#)



Eating what works for you

People who emerge from Bryan Health or CHI Health St E's after ostomy surgery might ask, "What can I eat now?"

The United Ostomy Associations of America (UOAA) notes that there is no single "ostomy diet." Instead, food tolerance is individual, and finding what works often takes a cautious trial and observation.

UOAA says introduce foods gradually, particularly after surgery. Try one new food at a time and pay attention to how your body responds. A simple food-and-output journal can help identify foods that cause excess gas, changes in output, or discomfort.

Pay attention to hydration. Because the colon normally helps absorb water, people without a functioning colon can lose more fluid through their stoma. UOAA recommends making hydration a regular part of the day and discussing your individual fluid needs with a WOCN.

Chewing is another simple but important habit. Some high-fiber or difficult-to-digest foods can increase the risk of blockage, particularly for people with an ileostomy. UOAA says eat slowly, chew food thoroughly, and be cautious when introducing foods such as nuts, popcorn, dried fruits, coconut, celery, mushrooms, and raw or crunchy vegetables.

Most importantly, don't let an ostomy convince you that food has to stop being enjoyable. Your diet may change somewhat, especially during recovery, but many people with ostomies eventually return to a broad and satisfying variety of foods.

If you have questions about a particular food, output change, or hydration problem, your ostomy nurse or doctor can help.

Source: [United Ostomy Associations of America](#) and [GI Nutrition Foundation](#)



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89611	89511	2-1/8" (55mm)	12" (30cm)	5

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Key Features: CeraPlus™ Skin Barrier (extended wear), Integrated AF300™ Filter, Lock 'n Roll™ Microseal Closure

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8960	3/4" (19mm)	12" (30cm)	5	
8961	7/8" (22mm)	12" (30cm)	5	
8962	1" (25mm)	12" (30cm)	5	
8963	1-1/8" (29mm)	12" (30cm)	5	

***viewing option

CUT-TO-FIT FULL HYDROCOLLOID BARRIER

Ultra-Clear Stock No.	Beige™ Stock No.	For Stomas Up To	Pouch Length	Box Qty
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8195411	8195111	2-1/8" (55mm)	12" (30cm)	5

HCPCS A5057
Key Features: CeraPlus™ Skin Barrier (extended wear), Integrated AF300™ Filter, Lock 'n Roll™ Microseal Closure

One-Piece Soft Convex Urostomy

CUT-TO-FIT WITH ADHESIVE BORDER				
Ultra-Clear Stock No.	For Stomas Up To	Pouch Length	Box Qty	
84134	1" (25mm)	9" (23cm)	5	
84138	1-1/2" (38mm)	9" (23cm)	5	
841311	2-1/8" (55mm)	9" (23cm)	5	

HCPCS A4430
Key Features: CeraPlus™ Skin Barrier (extended wear), Anti-Reflux Valve, Multi-Chamber Design, Soft Streamlined Tap, Drain Valve Indicator, includes one adaptor

PRE-SIZED WITH ADHESIVE BORDER				
Stock No.	Barrier Opening	Pouch Length	Box Qty	
8412	3/4" (19mm)	9" (23cm)	5	
8413	1-1/4" (44mm)	9" (23cm)	5	
8414	1" (25mm)	9" (23cm)	5	
8415	1-1/8" (29mm)	9" (23cm)	5	

***viewing option

Two-Piece Soft Convex Skin Barriers with Integrated Floating Flange

CUT-TO-FIT WITH ADHESIVE BORDER				
Stock No.	Flange Size	For Stomas Up To	Box Qty	
11702	1-3/4" (44mm)	1" (25mm)	5	
11703	2-1/4" (67mm)	1-1/2" (38mm)	5	
11704	2-3/4" (70mm)	2" (51mm)	5	

HCPCS A4407
Key Features: CeraPlus™ Skin Barrier (extended wear), Flexible design, Floating Flange

PRE-SIZED WITH ADHESIVE BORDER				
Stock No.	Flange Size	Barrier Opening	Box Qty	
11903	1-3/4" (44mm)	7/8" (22mm)	5	
11904	2-1/4" (67mm)	1" (25mm)	5	
11905	2-1/4" (67mm)	1-1/8" (29mm)	5	
11906	2-1/4" (67mm)	1-1/4" (32mm)	5	

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—Theresa K., WOCN

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Lincoln Ostomy Home

LOA welcomes calls from ostomates, especially new people, with questions on everything from, "how do I shower?" to "what about clothes?" Start with an email lincolnostomy@gmail.com

Meetings (Let's meet, shall we?)

In person meetings are at a church at 8230 South St., starting at 2 p.m. updates at ostomynebraska.com/lincoln
September 13 & December 6
See Page 2

Contact, Advertise

Contact or visit LOA to learn more about support group meetings, ostomy products and educational opportunities. Sparrow is a quarterly publication for the Lincoln Ostomy Association. If you'd like to advertise or to submit an article, email lincolnostomy@gmail.com

Ostomy Nurse Contacts

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Riddle

What am I?
See page 7

I have a heart but never beat,
I have a bed but never sleep,
And doctors use me to help you feel better.



6

Convatec visits LOA

Andrew Kauffman, Convatec Territory Manager, drove hundreds of miles to meet with members of the Lincoln Ostomy Association at the June 7 meeting.

Kauffman discussed the latest ostomy technologies offered by Convatec including new one-piece soft convex ostomy system, Esteem Body™ with Leak Defense™ which adapts to the body with soft convexity and Moldable Technology™ Skin.

"Convatec is looking at a partnership with the ostomy community," Kauffman said. "We want to support our friends with information and advice whether you choose our products or not."

Kauffman introduced the free me+ program designed to do just that. Those that enroll get full access to ostomy specialists and nurses ready to answer questions. The program also opens members to free samples, a travel kit, and online education. Signing up for me+™ by calling 1-800-422-8811.

Kauffman has worked with Convatec for three years joining the company inspired by his father's history with colorectal cancer.

